coursera

Depressive and Bipolar Disorders

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Start Your Course Here

✓ A Welcome Video From Your Instructor
Video • 2 min

✓ Welcome to PsycLearn Essentials
Reading • 3 min

○ Survey: What Would You Like to See Next?
Reading • 2 min

✓ How to Earn Certificates
Reading • 2 min

Course Overview

✓ What's in Your Course
Reading • 5 min

Study Success in PsycLearn

✓ Study Tips for Success in PsycLearn Essentials
Reading • 5 min

Additional Course Information

✓ Additional Information
Reading • 10 min

✓ Representation in Research
Reading • 1 min

Module 2
Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders

✓ Focus of the Course
Reading • 2 min

✓ Classifying Psychopathology
Reading • 4 min

✓ Culture and Psychopathology
Reading • 4 min

✓ Culture and Intersectionality
Reading • 4 min

✓ Culture and Stigma
Reading • 4 min

○ Check Your Understanding: Culture and Mood Disorders
Practice Assignment • 5 min

Lesson 2: Introduction to Depressive and Bipolar Disorders

✓ Depressive and Bipolar Disorders
Video • 6 min

✓ Living with a Mood Disorder: A Case Study
Reading • 10 min

○ **Overview of Depressive and Bipolar Disorders**
Reading • 10 min

○ Check Your Understanding: Introduction to Depressive and Bipolar Disorders
Practice Assignment • 5 min

Key Takeaways

○ Key Takeaways: Opening to Depressive and Bipolar Disorders
Reading • 10 min

Module 2 Quiz

○ Module Quiz: Opening to Depressive and Bipolar Disorders
Graded Assignment • 10 min

Module 3
Depressive and Bipolar Disorders

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

Module 6
Treatments for Depressive and Bipolar Disorders

Module 7
Suicide

Module 8
Conclusion

Module 9
Cumulative Quiz

Module 10
Course Resources

Module 11
PsycLearn Essentials APA Student Resources

Overview of Depressive and Bipolar Disorders

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It is estimated that 3.8% of the world population (more than 300 million people worldwide) are living with a depressive disorder (World Health Organization; 2023) and 1% of the world population is living with a bipolar disorder (Moreira, et al., 2017), indicating that mood disorders are a global health problem. Although effective treatments are available, more than 75% of low- and middle-income people diagnosed with these disorders receive no treatment (World Health Organization, 2023).

Episodes of depression are part of two separate categories of mood disorders: depressive disorders and bipolar disorders. Depressive disorders are sometimes referred to as unipolar depressive disorders to distinguish them from bipolar disorders. The word "bipolar" suggests two ends of a continuum or two extremes, in this case mood with depressed mood at one end of the continuum and mania or hypomania at the other end of the continuum.

The descriptions and diagnostic criteria for these two categories of psychological disorders are adapted from the International Classification of Diseases Eleventh Revision (*ICD-11*).

Depressive Disorders

There are several types of depressive disorders and all share three symptoms: negative mood, maladaptive cognition, and physiological changes. Negative mood can be characterized by persistent sadness, or **anhedonia**, and feelings of hopelessness. Maladaptive cognition can be characterized by feelings of worthlessness and failure, inability to concentrate, and thoughts of suicide and self-harm. Physiological changes can be characterized by poor or increased appetite, significant weight loss or gain, sleep disruptions, chronic fatigue, and a weakened immune system.

Depressive disorders can occur with no discernable environmental trigger, as a result of substance/medication use, or be brought on by another medical condition, such as pregnancy. Depressive disorders can also be differentiated based on specificity of the cause of the depressive symptoms and uniqueness of the symptoms not shared by another depressive disorder.

Of the different depressive disorders, these are the most commonly diagnosed:

- **Depressive disorders**: These are a spectrum of mood disorders that can be distinguished based on whether episodes are singular or recurrent, whether episodes are mild, moderate, or severe, and whether episodes include psychotic symptoms or not. Included within this spectrum of depressive disorders is recurrent depressive disorder with a seasonal pattern of mood episode onset, commonly referred to as **seasonal affective disorder**, which is diagnosed by depressive symptoms with a distinct seasonal pattern that is not attributable to a recurrent social event, such as school schedule or seasonal unemployment, but rather to changes in the seasons.
- **Dysthymic disorder**: Individuals diagnosed with this disorder have depressive symptoms that last for more than two years, without any significant break. Though the symptoms are experienced for a longer period of time, they are less severe than those associated with one of the spectrum of depressive disorders. This disorder is called persistent depressive disorder in *DSM-5*.
- **Premenstrual dysphoric disorder**: This disorder is associated with the menstrual cycle and symptoms include depression and irritability, physical symptoms, such as lethargy and pain, or cognitive symptoms, such as difficulty concentrating.
- **Mental or behavioral disorders associated with pregnancy, childbirth, or the puerperium**: This disorder, commonly called **postpartum depressive disorder**, is associated with pregnancy or the puerperium (also called the postpartum), which is the 6 week period following birth when the gestational mother's reproductive organs return to a nonpregnancy condition. This disorder is more accurately called **peripartum depressive disorder** because patients may experience symptoms prior to the birth of the child. Symptoms include mental and behavioral symptoms that may or may not include psychosis.

Bipolar Disorders

The symptoms for depressive disorders involve only those associated with depressed mood, at one end of the mood continuum. The other end of the mood continuum is **mania**, elevated, expansive, or irritable mood, often including symptoms such as an increase in activity; talkativeness or racing thoughts; inflated self-esteem or grandiosity; a decreased need for sleep; extreme distractibility; and intense pursuit of activities that are likely to have unfortunate consequences. **Hypomania** is a milder form of mania that does not include symptoms severe enough to impair one's job or relationships. **Bipolar disorders** are a group of disorders where symptoms of depression and mania or hypomania typically alternate, or cycle, although one episode of mania is enough for a diagnosis of bipolar I disorder. **Rapid cycling** is diagnosed with four or more cycles happening within a 12 month period.

The *ICD-11* and *DSM-5* list several types of bipolar disorders, which vary in terms of cause, or length, or severity of symptoms. They can be distinguished based on whether episodes are mild, moderate, or severe, and whether episodes include psychotic symptoms or not.

Of the different bipolar disorders, three are more commonly diagnosed:

- **Bipolar I disorder**: Individuals with bipolar I disorder fluctuate between depressive episodes and episodes of mania or hypomania, each of which can last for several days.
- **Bipolar II disorder**: A version of bipolar disorder in which the individual fluctuates between major depressive and hypomanic episodes, each of which can last for several days.
- **Cyclothymic disorder**: A mood disorder characterized by periods of hypomanic symptoms and periods of depressive symptoms that are not elevated to a depressive or bipolar episode but that occur over the course of at least two years.

In terms of living with bipolar disorder, on one hand, bipolar disorder is associated with creativity and professional accomplishment (Jamison, 2011; McIntyre et al., 2020), but on the other hand, the toll it takes on individuals and family members is devastating. Most people diagnosed with bipolar disorder experience significant distress and disability (McIntyre et al., 2020). Further, mortality studies find that bipolar disorder is associated with the loss of 10-30 potential years of life (Hansen et al., 2019; Kessing et al., 2015) most often as the result of suicide (Hällgren et al., 2019) and cardiovascular disease (Dhar & Barton, 2016; Kessing et al., 2015; Westman et al., 2013).

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